

|                                |                                           |                              |
|--------------------------------|-------------------------------------------|------------------------------|
| Name : <b>Saroj Agarwal</b>    | Age/Gender : 63 y / F                     | Collected : 21/02/2026 19:04 |
| Patient ID : <b>0010663186</b> | Visit Type : IP                           | Received : 21/02/2026 22:25  |
| Referred By : Dr. Darshit Shah | DOB : 27/08/1962                          | Reported : 24/02/2026 10:32  |
| Accession : 2600042126         | Location : 1T06N`1TR0601`1TB0605`1ONCOMED |                              |

**DEPARTMENT OF LABORATORY MEDICINE - MICROBIOLOGY**

**Test Name**

**Result**



**Sputum Culture**

|                       |                        |
|-----------------------|------------------------|
| Clinical Details      | Ca endometrium         |
| Sample                | Sputum                 |
| Antimicrobial Therapy | Inj Viatran 3000 mg BD |
| Report status         | Final Report           |

**Gram Smear**

|                       |                                                                                                |
|-----------------------|------------------------------------------------------------------------------------------------|
| Pus Cells             | >25/LPF                                                                                        |
| Epithelial cells      | Occasional                                                                                     |
| Gram Positive cocci   | Few                                                                                            |
| Gram Negative bacilli | Few                                                                                            |
| Yeast                 | Few                                                                                            |
| Remarks               | Gram Positive cocci in pairs and clusters<br>Yeast: Budding yeast cells seen with pseudohyphae |

**AFB Smear**

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Acid fast bacilli

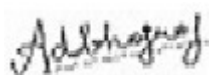
Not seen

Culture

No Classical Respiratory Tract Pathogen  
Isolated

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**Test Name Result**

**Sputum Culture:**

**Interpretation for Sputum specimen only:**

Grading of AFB smear by Z-N- Stain on basis of NTEP/WHO (National Tuberculosis Elimination Programme/ World Health Organisation) recommendation.

| Examination finding                      | No. of fields examined | Grading | Result   |
|------------------------------------------|------------------------|---------|----------|
| No AFB in 100 oil immersion fields       | 100                    | 0       | Negative |
| 1-9 AFB per 100 oil immersion fields     | 100                    | Scanty* | Positive |
| 10-99 AFB per 100 oil immersion fields   | 100                    | 1+      | Positive |
| 1-10 AFB per oil immersion field         | 50                     | 2+      | Positive |
| More than 10 AFB per oil Immersion field | 20                     | 3+      | Positive |

\*Record actual number of bacilli seen in 100 fields – e.g. "Scanty 4"

Method: Specimen processed on culture media. Identification by automated MALDITOF / Vitek2 & or conventional methods, sensitivity on Vitek 2 Certain antibiotics tested by Kirby Bauer's method and MIC value is not reported for antibiotics which are deduced by software as per CLSI Guidelines. Colistin MIC is tested by Microbroth Dilution method.

**Remarks:**

- Results of culture and antimicrobial susceptibility test needs to be correlate clinically.
- Previous history of antibiotic usage may influence the growth of microorganisms in vitro.
- Result is dependent on the quality of specimen submitted.
- Smear-positivity and grade indicates relative bacterial burden.
- Changes in smear status important for monitoring response to therapy.
- Kindly note AFB stain does not distinguish between viable and dead organisms nor differentiates species of Mycobacteria.
- Mycobacteria Genxpert and MTB automated culture recommended if clinically indicated.
- A negative AFB smear may mean that no infection is present, that symptoms are caused by some other microorganism, or that the mycobacteria were not present in sufficient numbers to be seen under the microscope.

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∞ End of Report ∞